Executive Summary

Care Transition Efficiency & Placement Outcome Analytics — Unaccompanied Alien Children (UAC) Program

Purpose

This summary presents findings from an analysis of daily public reporting data for the Unaccompanied Alien Children (UAC) Program, covering January 2023 through December 2025. The analysis moves beyond simple headcounts of children in custody and instead measures how efficiently children move through the care pipeline — from CBP custody, to HHS care, to reunification with a sponsor. The goal is to surface process risks that are invisible in capacity counts alone, so that program leadership can act on them directly.

Headline Finding

System volume fell sharply beginning in February 2025 — but this reduction in caseload did not come with a more efficient or more predictable pipeline. In fact, the speed and reliability of the pipeline both declined during this period. Lower volume and better performance are not the same thing, and the data shows the program should not assume one implies the other.

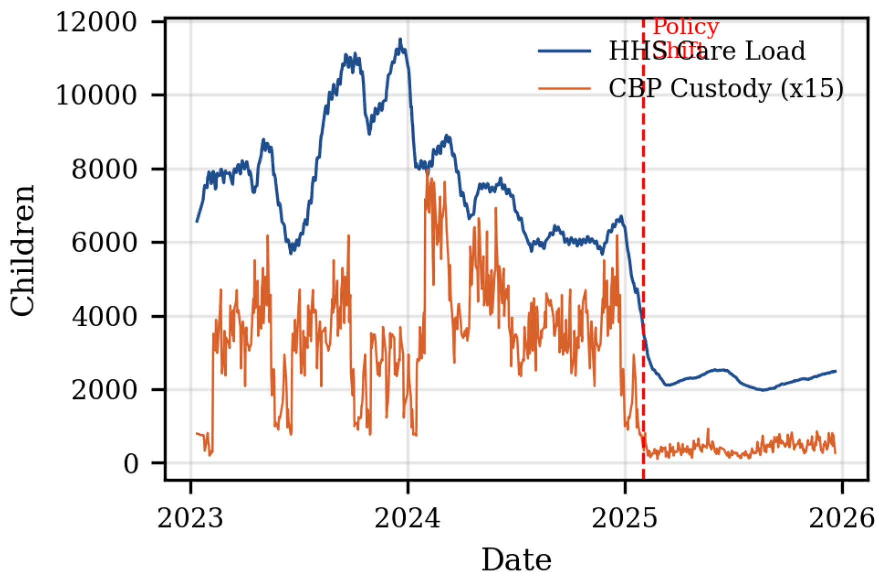

Figure 1: HHS care load and CBP custody, January 2023 – December 2025. The dashed line marks the February 2025 shift.

Key Metrics: Before vs. After the February 2025 Shift

Metric	Pre-2025	Post-2025	Change	What it Means
Average Children in HHS Care	7,686	2,295	**-70.1%**	System size shrank sharply
Average Children in CBP Custody	234	27	**-88.4%**	Border-side custody nearly emptied
Transfer Efficiency (CBP → HHS speed)	0.811	0.414	**-48.9%**	Transfers to HHS slowed by half
Discharge Effectiveness (reunification rate)	0.031	0.007	**-78.1%**	Reunifications nearly stopped
Outcome Stability (consistency of outcomes)	0.817	0.703	**-13.9%**	Outcomes became less predictable

What the Data Shows

- **Efficiency declined even as volume fell.** Total system size dropped by roughly 70%, but the speed of transfers and reunifications fell even faster — by 49% and 78% respectively. A smaller system did not become a faster one.
- **Outcomes became less predictable, not more.** Despite handling fewer children, the consistency of discharge outcomes worsened by 14%. This points to an operational issue, not simply a change in caseload.
- **Only one sustained bottleneck was detected in three years of data** — a five-day period from March 30 to April 3, 2025, in the smaller post-shift system. The larger, higher-volume system prior to 2025 was consistently able to clear cases faster than they arrived.
- **The program does not operate on Saturdays.** Sunday processing shows the highest average reunification rate, consistent with cases held over the weekend being cleared as the work-week resumes.

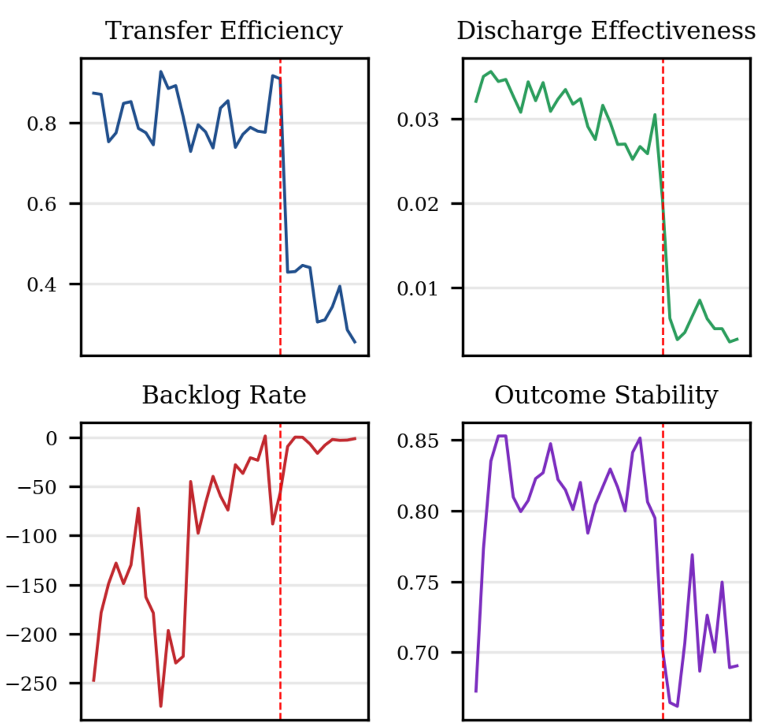

Figure 2: Monthly trends across four core efficiency indicators, January 2023 – December 2025.

Recommendations

- **Investigate the post-shift efficiency drop directly.** Determine whether staffing, screening requirements, or sponsor-vetting steps changed in proportion to the reduced caseload, since the data shows per-case friction increased rather than decreased.
- **Track efficiency and volume as separate indicators going forward.** Continue monitoring the five KPIs introduced in this analysis alongside raw headcounts, so future capacity changes are not mistaken for performance improvements.
- **Review the March–April 2025 bottleneck period** to identify its operational cause, since it is the only sustained backlog detected across three years and occurred in the lower-volume environment where such delays were not expected.
- **Use the accompanying interactive dashboard** for ongoing monitoring: it allows staff to filter by date range and policy period, and flags bottleneck periods and stability drops automatically as new data arrives.

Data Source: U.S. Department of Health and Human Services — UAC Program Public Reporting Dataset (January 2023 – December 2025). Full methodology, statistical detail, and figures are available in the accompanying research paper.